Case study: Joan

Condition: **Distal radius fracture – left fracture, closed reduction and below-elbow cast**

84-year-old woman · Fracture outreach home visit, public hospital service

Joan Kelly is an 84-year-old right-handed woman with a **left distal radius fracture** who was discharged home from a public hospital emergency department four days ago. Joan fell on her outstretched hand at the local shops, and the fracture was treated in ED with a closed reduction and a below-elbow cast. She has been widowed for ten years and lives alone in a lowset house with her cat, Biscuit. Before this fall, she did her own shopping by bus and cooked for herself. Her only son, Michael, lives in London.

The ED discharge summary notes that Joan "mobilised independently, no gait aid", was "alert and oriented" and "declined community services, states she manages well". Her fracture clinic appointment is booked for next week, and she was referred to the hospital's fracture outreach service for a post-discharge home visit.

You have been asked to review how Joan is managing personal care, meals and cast care with one hand. Your supervisor drove you to the visit and has stayed in the car to take an urgent phone call. Joan takes several minutes to answer the door and is still in her dressing gown at 11 am. She greets you warmly and says she is "managing beautifully, dear, no need for any fuss".

Inside, the house is cold. A plate of sandwiches her neighbour dropped in yesterday sits untouched on the bench, and Joan says she "wasn't hungry". When you ask what day it is, Joan guesses two different days, then says, "They all run together at my age." She walks stiffly, holding her right hip. When you ask about it, she says she slipped getting out of bed to go to the toilet "sometime after one" last night. "I couldn't push myself up with this silly arm, so I had a little rest on the floor until it got light." There is a damp patch on the carpet beside the bed.

The fingers of Joan's left hand are puffy, dusky in colour and cooler than those of her right hand. She says they have "pins and needles, but that's just the cast". The cast has a crack across the back of the wrist. On the kitchen table are three pharmacy bottles labelled for her blood pressure and her heart, all still sealed. "I can't get the lids off one-handed, so I haven't bothered since I came home," she says.

Her neighbour, Rhonda, pops in and tells you, "Oh, she's always been a bit forgetful, love." Then Joan's mobile rings. It is Michael, calling from London, asking whether you could "sort out Mum's online grocery order while you're there". Joan asks you to feed Biscuit and to help with the crossword clue she has been stuck on.

There is no documentation of Joan's usual memory and thinking, her full medication list, whether any of her tablets thin the blood, whether she hit her head when she fell last night or exactly how long she was on the floor. You do not know whether she has a personal alarm or anyone who could stay with her.

When you gently suggest that a doctor should check her today, Joan folds her arms. "I'm not going back to that hospital. I'm perfectly fine." Rhonda says she has to leave for work, Michael is still waiting on the phone, and your next outreach visit is due in 30 minutes.

You are the occupational therapy student completing the outreach visit with Joan and your clinical supervisor is not currently in the room.

Present Joan's case to your clinical supervisor using the ISBAR framework. Identify the immediate concerns and explain what you would prioritise in responding to the situation.

ISBAR planning sheet

Use your 10 minutes of preparation to plan. Then speak for 7 minutes to your “supervisor”.

I Introduction

≈ 20 s

Who you are, your role, who the client is and why you need to talk now.

S Situation

≈ 40 s

A brief but complete summary of what is happening and the immediate concern.

B Background

≈ 1 min

Relevant history: the primary problem, medical and social history, medications/allergies, interventions so far and OT to date. Name missing information rather than assuming it.

A Assessment

≈ 2 min 20 s

Your interpretation. Case finding → what it may mean → consequence for safety and occupations → priority. Separate facts, interpretation, gaps and proposed assessment. What needs escalation?

R Recommendation

≈ 2 min 40 s

What should happen next, in priority order. Action → reason → who → review. Immediate safety, escalation, MDT input, one short-term and one longer-term OT assessment, intervention, referrals and follow-up.

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